NCCP 00448 · clinician draft

# Panitumumab + FOLFIRI

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Panitumumab + FOLFIRI

**Active medicines:** panitumumab + irinotecan + folinic acid + fluorouracil

**Emetogenic risk:** moderate

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 16 mg orally | Day 1 before chemotherapy<br>Dose may be adjusted for individual/QT risk. |
| Dexamethasone | 8 mg orally daily | Days 1–3<br>Reconcile all steroid prescriptions. |
| Metoclopramide | 10 mg orally up to three times daily as prescribed | As required on subsequent days<br>Confirm suitability, duration and contraindications. |
| Atropine | 250 micrograms subcutaneously | Before irinotecan per NCCP prophylaxis<br>Check contraindications including glaucoma; this does not treat delayed diarrhoea. Use caution in asthma, as specified in the NCCP acute cholinergic-syndrome advice. |

### Additional phase-specific support

- No routine panitumumab premedication unless previous hypersensitivity/local policy indicates.
- Written delayed-diarrhoea plan and urgent fever/dehydration advice; do not confuse with acute cholinergic management.
- Skin care and electrolytes; monthly Mg/Ca/electrolytes for two months after last panitumumab.
- No routine panitumumab premedication unless previous hypersensitivity/local policy indicates. Written delayed-diarrhoea plan and urgent fever/dehydration advice; do not confuse with acute cholinergic management. Skin care and electrolytes; monthly Mg/Ca/electrolytes for two months after last panitumumab.
- Supply a written irinotecan delayed-diarrhoea rescue prescription: NCCP reference loperamide 4 mg initially, then 2 mg every 2 hours; continue for 12 hours after the last liquid stool, never longer than 48 hours at this high-dose schedule. Start advised electrolyte-containing fluids at the first liquid stool and contact the team early. Fever, dehydration, severe pain or persistent symptoms require urgent oncology review.
- Irinotecan treatment-related diarrhoea must fully resolve before restart; a table-grade threshold alone does not clear the next dose.
- If treatment is delayed for cytopenia, follow the source-specific weekly blood-count checks and maximum delay; assess the antibody and chemotherapy components separately when indicated.
- Medication review: NCCP contraindicates concomitant brivudine, sorivudine and analogues with fluorouracil; review DPD-affecting medicines, phenytoin toxicity/levels and warfarin INR.
- Review CYP3A inducers/inhibitors and grapefruit; avoid prochlorperazine on the same day as irinotecan. Gilbert syndrome warrants consideration of a reduced starting irinotecan dose; no universal percentage is supplied.
- EMA Vectibix also requires interruption for intolerable dermatologic reactions; a grade below 3 does not automatically clear treatment.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00448 v5 · p6 · NCCP NCIS Medical Oncology antiemetics V8, Table 1

Sources checked 2026-11-05 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required